

The All-in-One Diagnostic Lab Platform

One portal to run the entire lab — offline-resilient,
transparently priced, ABDM & NABL ready. Built for India's
Tier 2 & Tier 3 labs.

Works offline

Compliant B2B & analytics

WhatsApp reports

India-sovereign & compliant

A large, fragmented market — formalizing fast



India's diagnostic-lab market is huge and overwhelmingly unorganized — and NABL accreditation, the ABDM digital mission and the DPDP privacy law are pushing labs to digitize right now. That formalization is exactly the trigger that makes fragmented SMBs adopt SaaS.

130k+

Diagnostic labs in India
Highly fragmented market

~82%

Unorganized
Standalone & small chains

\$11B

Market size (2025)
Growing ~11–12% CAGR

~75%

of TAT & errors
are non-analytical

The opening: incumbents are either premium-but-expensive and internet-dependent, or cheap-but-shallow. The **affordable-yet-complete middle** — with India-specific workflows and offline resilience — is wide open.

Where labs lose money & time today



B2B revenue under-managed

Institutional receivables run 90+ days overdue and per-partner rate lists are manual. (Paying referral commissions is illegal — DiagDesk does it the compliant way.)



Turnaround & errors

~56% of TAT is pre-analytical; 5.15% of samples are rejected from mislabeling and transcription.



Cash & billing leakage

Manual registration, paper dues, untracked discounts and unbilled add-on tests drain revenue.



Patient experience

Patients expect WhatsApp reports & app booking; D2C brands take footfall on price and speed.



Compliance burden

NABL QC, DPDP, CERT-In, PC-PNDT and CGHS rules are rising — and increasingly non-optional.



Multi-branch fragmentation

Collection centers and hubs run as disconnected islands; reagents stock out while overstock expires.

One portal to run the entire lab



DiagDesk replaces the stitched-together stack with a single, all-in-one portal — and it keeps working when the internet drops.



Offline-first

Registration, billing, barcoding & results run at the counter during outages, then sync.



Compliant B2B revenue

Institutional billing, B2B credit & receivables, rate cards, referral analytics — no illegal commissions.



Patient experience

Online booking, home collection, WhatsApp/SMS/email reports, online payments.



Compliance by design

NABL-ready QC, DPDP, PC-PNDT Form-F, ABDM — built in, not bolted on.

Core Lab Ops
Downtown Branch

Patient Registration Works Offline

Search recent patients...

Demographics ID: LUM-2023-9942

Full Name
John Doe

Age
28

Gender
Male

Phone Number
+91 98765 43210

Referring Doctor
Dr. Sarah Mitchell

Test Selection

Search tests (e.g. CBC, Lipid Profile, HbA1c)...

Common Cardiology Diabetic Hormones

Complete Blood Count (CBC)
Hematology • 4 hours ₹450 +

Lipid Profile
Biochemistry • 8 hours fasting ₹850 +

HbA1c (Glycosylated Hemoglobin)
Diabetic Care • 6 hours ₹600 +

Cart Summary 2 Items

Complete Blood Count (CBC)
Hematology ₹450

Lipid Profile
Biochemistry ₹850

Subtotal ₹1300

Discount -₹100

Total ₹1200

PAYMENT METHOD

Cash UPI Partial

Due Balance ₹0.00

Confirm & Generate Receipt

Peak Hours Active
Estimated TAT: 4.2 Hours

+ New Patient

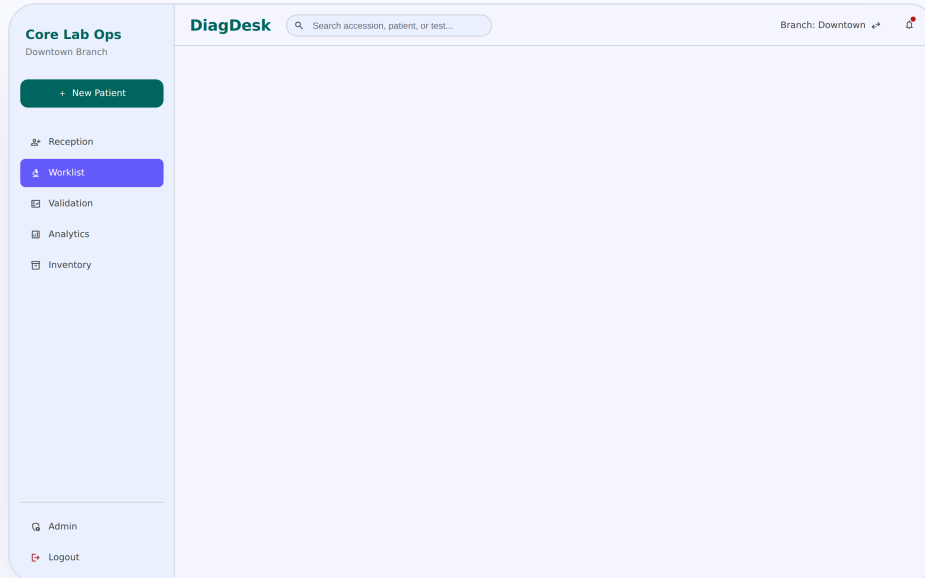
Admin

Logout

Reception & billing — works offline, GST-aware, cash/UPI/partial



Register → track → validate → report

**DiagDesk**

Barcode sample tracking

Accession to report, with rejection reason codes feeding QC.



TAT dashboards & alerts

Pending-beyond-TAT samples surfaced before they breach.



Analyzer interfacing

HL7/ASTM kills manual transcription across major analyzers.



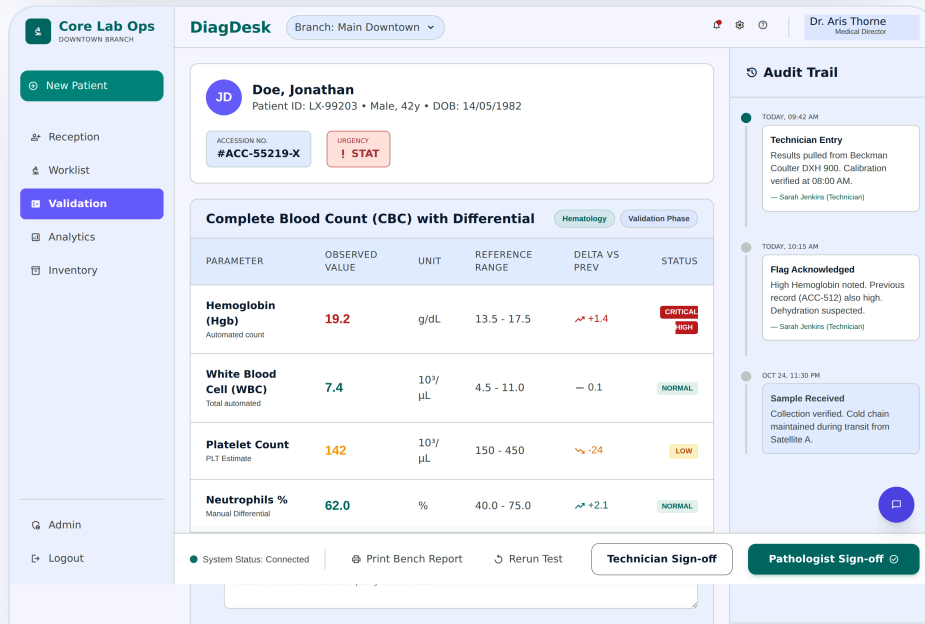
Multi-level validation

Reference-range flags, delta checks, pathologist sign-off.



Letterheaded, signed reports

Your saved letterhead, an inline-editable preview, then owner/pathologist review & digital signature.



Billing & B2B — the compliant India edge



Paying referring doctors a commission is **illegal** in India (and binds the lab as payer). DiagDesk handles the **legal** money flows — institutional B2B billing, referral analytics, and **pay-for-genuine-services contracts** (a consultant read, not a referral cut) — where incumbents risk it with kickback modules.



Flexible billing

Cash / UPI / partial payments, dues tracking, GST-aware invoicing, day-end reconciliation.



B2B credit & receivables

Institution-as-buyer accounts, billing cycles, aging dashboards and overdue alerts.



Referral analytics + doctor portal

Track referral sources & engage doctors with reports — analytics only, never a payout.



Rate-card manager

Per branch, per partner, per scheme — CGHS TMS 2.0, ECHS, TPA.

100%

legal money flows
no kickback tooling

90+ days

typical B2B overdue
aged & chased in-app

1 ledger

cash, B2B credit & receivables
no more spreadsheets

Day-to-day control, the way you run it



Shaped by a working lab owner's wishlist — the controls that decide whether software actually fits the floor: who can do what, what stock you have, where the money goes, and proof the report reached the patient.



Granular roles you set

Decide each user's exact access at account creation — discounts, rate edits, which financial reports, date changes — with per-user limits.



Inventory & test-kits

Track kits by **tests-per-kit** (10/100...), auto-deduct on each test, and get **low-stock alerts** at your reorder threshold.



Expense management

Log day-to-day expenses by category — rent, salaries, utilities, reagents, petty cash — with expense reports.



Discount control

Per-user discount limits with a **mandatory justification** on every discount; above-limit needs approval — all audit-logged.



Report handover & prints

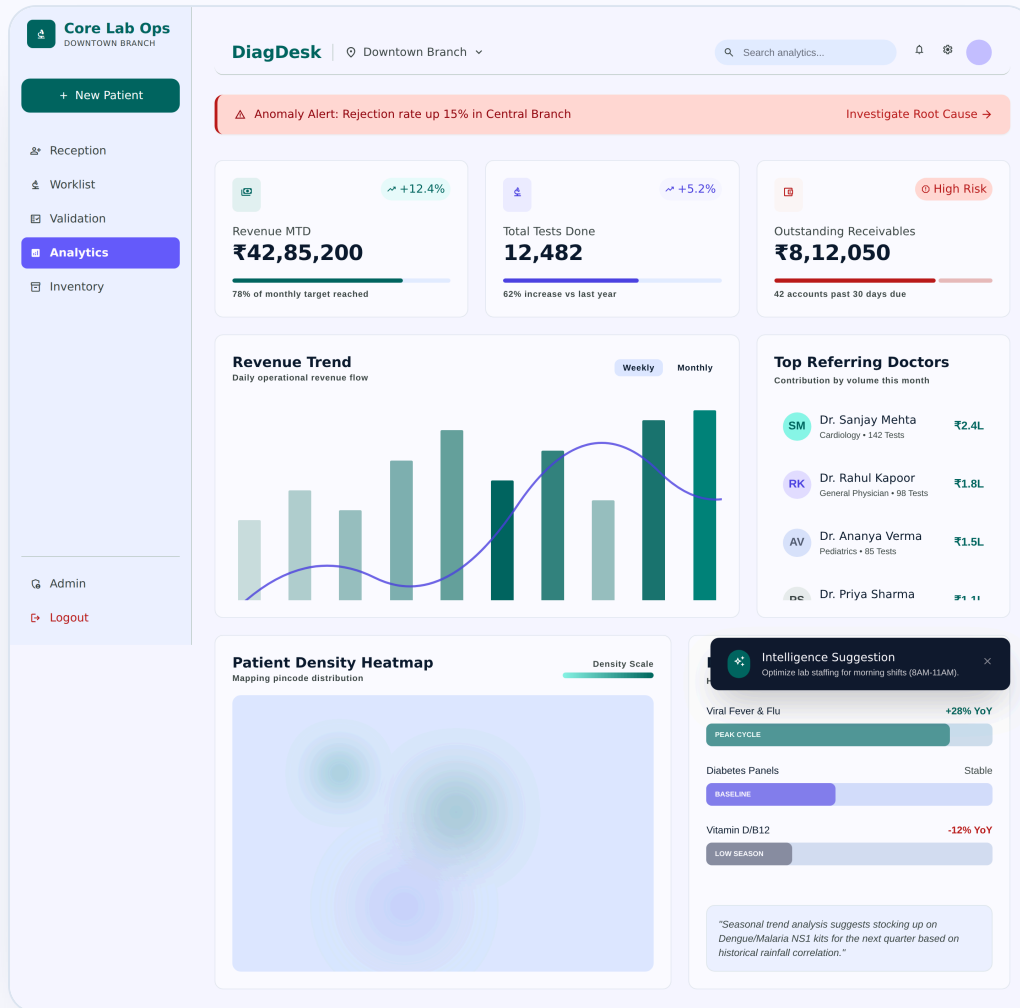
Confirm a report reached the patient by **scanning its barcode** at handover; see the **print count** per report.



Audit & backups

Complete per-patient, per-investigation audit trail; **local + cloud backups** so nothing is ever lost.

An AI-enriched owner command center



Open DiagDesk and know — in seconds — what's working, what's not, and what to act on.



Revenue & growth

By branch, test & doctor; cash vs credit; receivables risk.



Top referrers & win-back

Revenue generated by source & a call-list for declining doctors (analytics, no payouts).



Where patients come from

Geographic / pincode heatmap and channel mix.



Seasonality & alerts

Demand forecasts, anomaly alerts and daily digests on the go.

The right tool for each user



DiagDesk
Downtown Branch

Good morning, Dr. Sarah

Here is what's happening across your branches today.

TODAY'S REVENUE

\$12,840.00

↗ +12.4% from yesterday

COLLECTIONS VS DUES

82% Recovered

\$42k Collected \$51k Total Dues

Critical Alerts [View All](#)

Critical Rejection in Branch B 2m ago

Automated validation failed for 12 samples due to reagent batch #402 expiry.

Dr. Rao referrals down 40% 1h ago

DiagDesk

Hello, Sarah

Your Health Dashboard

Need a checkup?

Book a lab test or schedule a home collection in minutes.

[Book a Test →](#)

Report Ready NEW

Complete Blood Count (CBC) report is now available.

[PDF](#) [WhatsApp](#)

Upcoming

Tomorrow, Oct 24 Home Visit

08:30 AM

Full Body Screening
6 tests included

DiagDesk
Core Lab Ops • Downtown Branch

REMAINING

08 Visits

COMPLETED

04 Visits

Search patient or area...

Today's Visits Oct 24, 2023

IN TRANSIT

Rohan Malhotra **09:30 AM**

Complete Blood Count + Lipid Profile **Slot 1**

H-12, Sector 44, NOIDA, Uttar...

[Scan Doorstep](#) [Map Route](#)

UPCOMING

Sneha Kapoor **10:45 AM**

HbA1c + Vitamin D Check **Slot 2**

Apt 405, Green Tower, Golf Course

Built to beat the incumbents' weak spots



Offline-first resilience

The lab never stops — directly attacking the #1 complaint about cloud-only incumbents.

Differentiator



Transparent pricing

All-inclusive, published pricing — no surprise onboarding or per-integration fees.

Trust



Compliant doctor engagement

B2B billing + referral analytics + doctor portal — the legal relationship edge D2C brands ignore (no kickbacks).

India edge



One unified portal

LIS, billing, patient apps and (V2) radiology in one place — not five tools stitched together.

All-in-one



India-sovereign & compliant

Hosted in India (no global hyperscalers); DPDP, NABL, CERT-In and ABDM built in.

Compliance



Reliability & support

Stability SLAs and India-time support as a core promise — to win switchers.

Service

Grows with the lab



Radiology (RIS + PACS)

Unified path + imaging: modality worklist, structured reporting, light PACS & teleradiology.



ABDM & NHCX

ABHA linking and FHIR care-context to unlock incentives; digital insurance claims.



AI assists

Registration from handwritten requisitions, anomaly detection and demand forecasting.



APIs & booking widget

Public APIs, webhooks and an embeddable website booking widget.



Self-serve builders

Report and financial builders so labs customize without professional services.



NABL QC suite

IQC with Levey-Jennings & Westgard rules, EQAS logs and controlled documents.
(Inventory is now built into the core.)

Phased delivery — value early

MVP**Run the lab**

0–4 months

- Register → barcode → validate → report
- Billing + inventory + expenses
- Granular roles, letterhead & handover
- Offline-first + DPDP + backups

V1**India money & compliance**

4–8 months

- B2B + compliant referral economics
- NABL QC + health packages
- Booking + home collection + payments
- Analytics dashboards

V2**Unified diagnostics**

8–14 months

- Radiology RIS + light PACS
- ABDM incentives + NHCX
- Self-serve builders + APIs
- AI registration & forecasting

Beyond — V3 MediCircle: connect doctors, pharmacies, diagnostic centers & home-care around the patient, with DiagDesk as the first node. **Next step:** approve the plan and ship the first end-to-end "walk-in" workflow — a lab running fully on DiagDesk, offline-resilient, in weeks.